

Atezolizumab + bevacizumab

Advanced or unresectable hepatocellular carcinoma / NCCP 00831 v2a

Your medicines

Atezolizumab · Bevacizumab

Your treatment schedule

Every 21 days

A picture of side effects

Tell your team about new or worsening symptoms. The same symptom can have more than one cause. Numbers connect each symptom box to its location.

Atezolizumab · immune effects

Bevacizumab · vessels / healing

1 Hormones / brain

Atezolizumab

Tiredness, thirst, dizziness or a new headache (hormone or pituitary inflammation).

3 Heart

Atezolizumab

Chest pain, palpitations or shortness of breath (myocarditis / pericarditis).

5 Kidneys

Atezolizumab

Passing less urine, swelling or blood test changes (nephritis).
Bevacizumab can also cause protein in the urine, usually found on a urine test.

7 Skin / nerves / muscles

Atezolizumab

Rash or blisters; weakness, numbness or muscle pain (skin inflammation / nerve effects / myositis).

9 Blood vessels / healing

Bevacizumab

Bleeding, clots or slow wound healing (bevacizumab-related effects).

2 Lungs

Atezolizumab

New cough or shortness of breath (pneumonitis).

4 Liver

Atezolizumab

Yellow eyes, dark urine or changes in blood tests (hepatitis).

6 Bowel

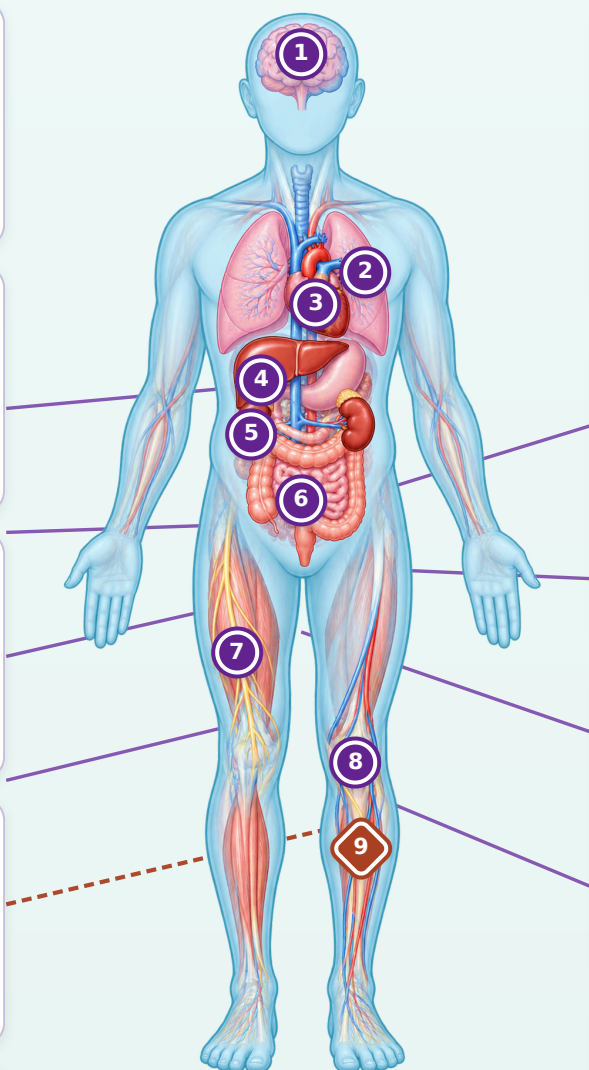
Atezolizumab

New diarrhoea, blood or tummy pain (colitis; severe pain may have another urgent cause). Bevacizumab can rarely cause a gut perforation; severe belly pain needs urgent assessment.

8 Joints

Atezolizumab

Painful, swollen joints (inflammatory arthritis).



This map shows selected effects to discuss; it does not show their likelihood or every possible side effect. Blood-count and whole-body effects are marked schematically.

Also report new jaw or tooth pain. Seek emergency assessment for a severe headache, vision change, confusion or seizure.

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Your treatment journey

Before C1

Varices screening; blood count, liver/kidney, glucose, thyroid, BP and urine protein assessment.

Days 2-21

Notice new symptoms. Follow any home BP instructions and keep your SOS number accessible.

Later / after

Each cycle follows the same 21-day pattern. Immune effects may still appear after treatment stops.

C1 · Day 1

Atezolizumab IV or SC, then bevacizumab IV. Tell the nurse immediately about an infusion or injection reaction.

C2 · Day 22

Repeat review, bloods, BP and urine; thyroid testing is scheduled every six weeks. Treatment only if appropriate.

Monitoring and support

- Before starting Bleeding and liver context Oesophageal varices screening, BP, urine protein, blood count, liver/kidney profile, glucose and thyroid tests. Discuss past bleeding, surgery, autoimmune illness and medicines with the team.
- Each cycle Blood, urine, BP NCCP specifies blood count, liver/kidney profile, glucose and urine-protein dipstick before each cycle, plus BP before and after treatment. Results are assessed alongside symptoms.
- Every 6 weeks Thyroid tests Thyroid function tests are scheduled every six weeks. Your team may order additional endocrine tests when symptoms suggest adrenal, pituitary or glucose problems.

04 / action language Know when to call Emergency: call 112/999 Collapse, severe bleeding or vomiting blood, severe chest pain, severe breathing difficulty, seizure, sudden new vision loss or a major neurological change. Tell responders you receive atezolizumab and bevacizumab. Contact oncology urgently Black stools, any concerning bleeding, severe or persistent diarrhoea, new cough or breathlessness, yellow eyes, reduced urine, new severe headache, new vision changes, marked weakness, confusion, a temperature $\geq 37.5^{\circ}\text{C}$ or $\leq 36^{\circ}\text{C}$, or feeling suddenly unwell. Follow your centre's SOS instructions even if a symptom begins after the last cycle. Medicines and fever Measure your temperature before paracetamol or ibuprofen if you feel unwell, and contact your team for advice. Do not self-start steroids for suspected immune toxicity. Ask before using anti-inflammatory painkillers, herbal products or new medicines, particularly if you have liver disease or bleeding risk. What about anti-sickness or a PPI? The NCCP lists both medicines as having minimal emetogenic potential and says premedication is not usually required. No routine antiemetic or PPI script is invented here. Continue medicines actually prescribed to you and ask for a reconciled list if you are unsure.

Scan to open your treatment portal

Regimen information only; this code contains no patient details.

<https://sactcheck.com/docs/patient/00831/>

